



An agent rather than a workspace. Arya picks up a referral, texts the patient to confirm discharge, ranks eligible clinicians with a match score and a suggested visit time, and writes the start of care back to HCHB once someone approves it. Clinicians work with it by text and short links, with no app. It is the most automated product in the round. It also needs the hardest look at its integration (Citrix screen automation, 15–20 minute reads) and at how its learned rules are governed. The capacity view we most need was promised but not shown.

SCORECARD GOING IN 84 Advance · no open stop-checks · highest in the field	LIVE FOOTPRINT ~60 orgs Per their return. Bayada, Pennant and Aveanna on the opening slide	GUIDE QUESTIONS 3 / 17 answered in full; 8 partly; 1 not answered; 5 not asked	NEXT TOUCHPOINT Follow-up Another virtual session before on-site selection; on-sites the week of 19 Oct
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Six things that matter

- 01
- It does the work, then asks.** “This is not another software for your office staff to log into... Arya does all the work... and escalates things it cannot schedule.” Much of it runs with no one approving each step: patient and clinician texts, outreach to eligible clinicians, and re-forming the week after a call-out. New-case staffing had an approval gate for the demo. Kunal: normally “the office staff is being told I am staffing a new case,” not asked. 0:25:27 1:25:27 1:28:29

- 02 **Every recommendation comes with a reason and a time.** “Quilla Lindqvist, 100% match, 1.6 mi, 18 pts, full-time,” with a slot of 1:42–2:42 pm chosen to keep her total drive time lowest. The slot sits inside a 24-hour window set from the patient’s own text. Hard *guardrails* decide who is eligible. Weighted *heuristics* decide who fits best. 0:29:18 0:36:15
- 03 **The clinician’s channel is text, not an app.** Preferences (days, times, regions) are captured by short link at onboarding. The next day’s route comes with drive time and a ready-made heads-up to the patient. A call-out is one text (“I can’t work any of these”). Clinicians can also phone in; a voice agent was offered afterwards but not shown. 0:42:18 0:55:17
- 04 **Patient outreach is live in the product.** It checks discharge and captures preferences by text, and claims Spanish text and voice. On Evan’s 30–60 minutes a day of clinician calls, Kunal said the work “should be close to zero, with the exception coming back to the clinician.” That is a strong claim with no measured result behind it. 1:09:25 1:12:13
- 05 **HCHB integration is Citrix screen automation.** Reads every 15–20 minutes and writes about every 15 minutes, if single sign-on doesn’t log the bot out every 20–30 minutes. They will load-test on our sandbox. The HCHB API is “supposedly coming in 2027.” Their return claimed latency under five minutes. 1:53:10 1:53:37
- 06 **Capacity planning was the gap.** Capacity windows are the core concept: 24 hours for SOC, the Medicare week for recurring visits, a fixed day for wound care. Auth-window escalation and “Put in review” holds were shown. The accept-or-decline view was not: “better if we come back on the capacity stuff... we don’t have good data.” Their RFP return fills in the design: a referral-level check of which SOC-eligible clinicians have room inside the window, and what the referral does to capacity over the episode, built from clinician capacity rolled up by branch. The return has no forecast of referrals or discharges and no leader dashboard. 1:45:53 1:47:33

SUGGESTED SCORECARD MOVEMENT

Hold at Advance. The closest thing to the model we are designing, and the one to test hardest.

Arya was the only vendor to run the whole loop live: referral, patient text, ranked clinicians, clinician confirmation, write-back to HCHB. The questions now are about the rails, not the idea. Can screen-automation refresh hold at our volume? Can each automated action stay behind approval permanently? Who governs the rules the agent learns from free-text feedback across 80+ branches?

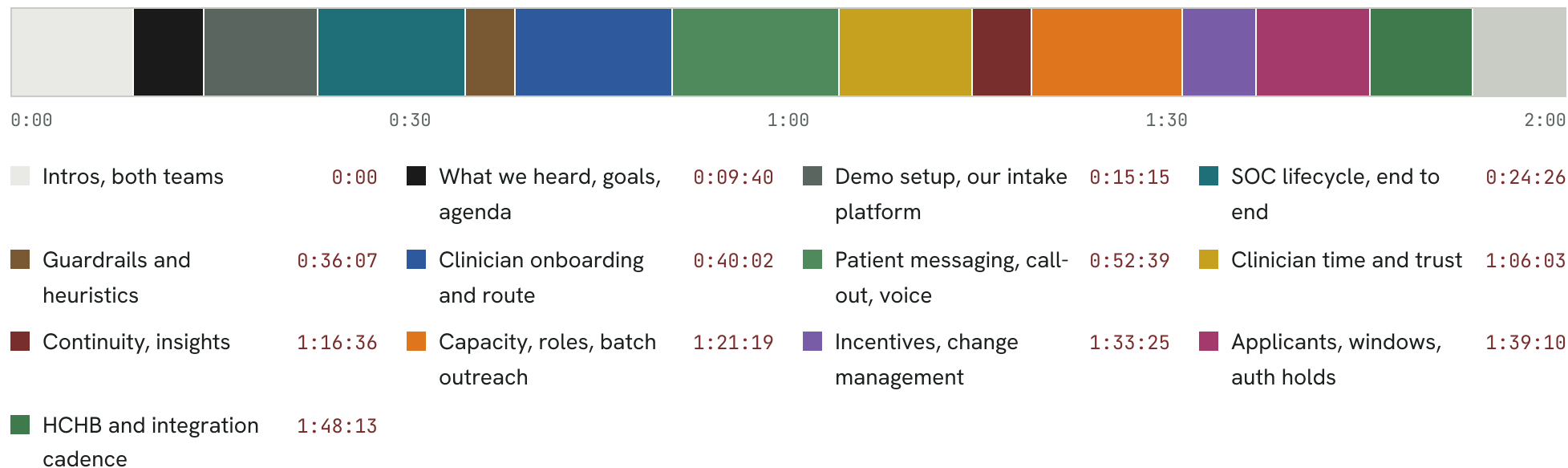
Take the follow-up session. Before the on-site, ask for three things: sandbox load-test numbers for reads and writes, the capacity view running on real data, and the rules (“the brain”) visualization they say they have just built.

Two parts are worth copying whoever we pick: collecting clinician preferences at onboarding and showing the reason next to every recommendation.

WHERE THE TWO HOURS WENT

PROPORTIONAL TO CLOCK TIME

The shape of the call



Arya ran a slide-framed demo that followed one start of care from referral to write-back. Our team then steered it with questions about the clinician's control, the business case and capacity. Integration came up only in the last eight minutes. Change management got the dedicated slot Arya had asked for.

What we heard



One HCHB instance, clinician-driven scheduling

Office schedulers plot visits from the frequency set at the initial assessment. Clinicians then arrange the day-to-day and week-to-week timing with patients, accounting for travel and personal availability.



Clinicians carry much of the coordination burden

Call and text patients to firm up visits, sometimes fielding replies at night to settle the next day's schedule. Estimated that roughly half of clinicians manage this well; the work depends on skills they were not hired to perform.



The information needed to schedule is scattered

PTO, on-call coverage and other availability live in Outlook calendars outside the EMR. Territories are largely drawn by local judgment and are difficult to adjust as demand moves. Branches lack an easy, current view of clinician capacity and suitable visits.



Changes require careful judgment and office intervention

A clinician may move their own visits after a disruption, but that can push work to the end of the week and constrain capacity.

Goals for automation

Any new scheduling approach must reduce manual burden, while preserving clinician input.

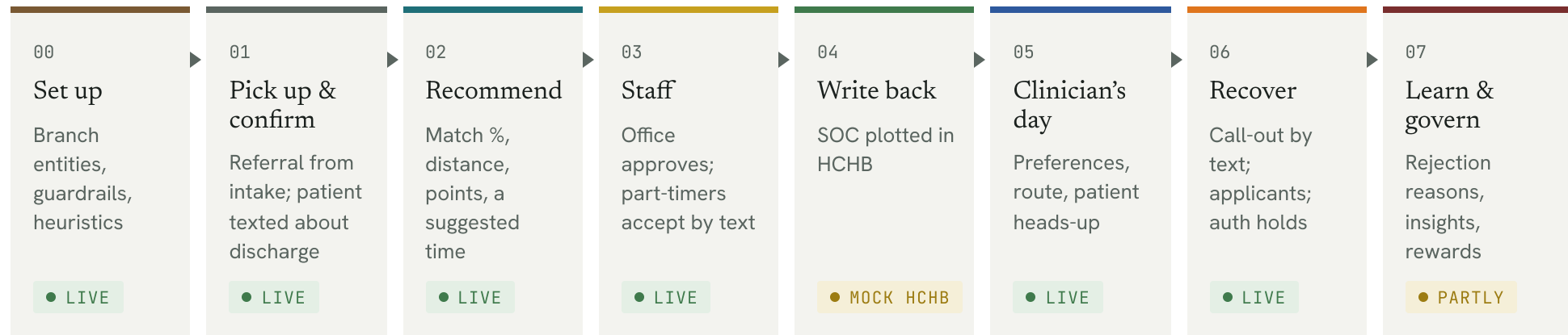
0:12:40 Their goal statement. Evan: "preserving the clinician input... is going to live or die on that" 0:12:46

Our position, as stated in the room

ITEM	AS STATED	WHEN
Intake platform	Built with an engineering partner. Its output is a “full, complete, valid referral,” with every check done except authorization. It was built to integrate with other tools. We would rather Arya plug in here than read from HCHB. Evan: “the more we can disintermediate HomeCare HomeBase, the better”	0:19:13 • 0:23:27
Capacity before accept	Michael: “That’s the problem we want to solve for.” Today it is branch spreadsheets, grids, a SharePoint site and Teams chats, checked by hand on every referral. This differs from the CareStitch call, where we said capacity at accept/decline was out of scope. Align before the on-sites	0:22:27 • 1:42:18
Discharge date	A referral isn’t pushed to HCHB until someone enters a confirmed discharge date	0:24:06
SOC timing	48 hours is the usual target. The goal of this effort is to admit within 24 hours	0:28:58
Scheduling model	Heading toward centralization (Evan). Arya suggested starting with weekends and after-hours	0:32:46 • 0:33:07
Clinician data	Some in HCHB, most in Workday. Territories are not mapped in HCHB; they live on office whiteboards, and tenured clinicians “have a lot more social influence” with schedulers	0:42:12 • 0:47:18
Biggest ask	Our schedulers are admin staff and clinicians do the real scheduling by phone, 30–60 minutes a day at licensed rates. Patients rarely answer calls from clinicians’ cell numbers; a Compassus number would help	1:09:25 • 0:53:39
AI with patients	Evan: “pretty forward leaning,” though clinical operating leaders “may reel us in just a little bit”	1:09:05
Timeline	On-sites the week of 19 October, after more virtual time with vendors	1:55:51

Pick up, confirm, recommend, staff, write back

Arya treats every piece of care as a *capacity window*: the span inside which a visit must happen. It then searches each eligible clinician's calendar for the best slot inside that window. The office and the clinician stay in the loop by approval or by text. HCHB stays the system of record, and Arya writes changes back to it.



● LIVE demonstrated in software ● PARTLY shown on a mock or with parts still being built

● NOT SHOWN the branch capacity view for accepting or declining a referral

Rules per branch, in two layers

WHAT IT DOES

- **Entities.** Each branch is an entity. The agent inherits rules set at the organization level, and each branch can override them for its own shortages, overtime, or rural and urban driving.
- **Guardrails** are hard requirements: licensure (an RN for a SOC), on-call and PTO calendars, a productivity-point ceiling, maximum travel time (e.g. 90 minutes), minimum time between visits, and hard patient preferences such as gender, smokers or pets.
- **Heuristics** rank the eligible clinicians: total drive time for the day, preferred days and hours, full-time before PRN, prior experience with the patient (last 30 days), and patient and caregiver preferences.
- The demo tenant: one mocked HCHB instance, five home health entities, 20 synthetic clinicians (RN and LVN, full- and part-time), and three visit types: SOC, standard recurring and wound care.

FOR COMPASSUS

- We decide which rules are hard and which are soft. “If you think clinicians should not have a say on which regions they go to, that becomes a hard requirement.” 0:36:15
- Branch-level override fits our branch variation. It also means 80+ rule sets that can drift apart; central ownership of the defaults is our job.
- Continuity counts among the heaviest heuristics, and Arya says it can be made a hard rule. It will then report how often no clinician qualifies (“agent health”). 1:16:36

“Guardrails are hard requirements that the agent must work inside of... heuristics is where the agent decides who is a better fit.”

Two core building blocks of the scheduling agent



0:36:21 The two building blocks: guardrails (hard rules) and heuristics (the mental model for trade-offs)

A starting point for Scheduling Agent: “Guardrails”

Guardrails

Hard requirements for the agent

- Time requirements
 - Not already working
 - Will not go over X productivity points
 - Minimum travel time between visits
- Max travel time (eg. 90 mins max)
- On-call and PTO calendars
- Skills and Licensure: RN vs LVN/ LPN
- Any other hard patient preferences
 - Areas they travel to
 - Gender (if communicated)
 - Are there smokers in the patients' home
 - Pets (if severe allergies noted for caregiver)

The mental model for trade-offs: “Heuristics”

Heuristics

Mental model for making trade-offs

- Total travel time for the day based on route
- Preferred schedule and preferred hours
- Full time vs PRN
- Prior experience with patient (last 30 days)
- Patient preferences
- Caregiver preferences
 - Eg. desired times of day/ hours
 - Desired hours staffed
 - Pets/ smoking etc

How we've setup the demo

1

Mocked HCHB instance

Full activity log of reads, writes & failures

5

Home Health entities

Anaheim, Irvine, Mission Viejo, Corona, Pomona

20

Synthetic clinicians

RN & LPN/LVN, full- and part-time, real constraints

3

Visit types seeded

SOC, standard recurring, wound-care recurring

Each Home Health entity is seeded with an unassigned SOC, an assigned recurring series, and a wound-care visit, so both open demand and existing workload are visible from the first screen.

THE THREE SCHEDULING WINDOWS CONFIGURED FOR THIS DEMO

Start of Care

24-hour operational window from the synthetic ready-to-staff timestamp

"Standard" recurring

Configured Medicare-week window for the synthetic episode

Wound-care recurring

Fixed weekday window

0:15:15 The demo tenant: 1 mocked HCHB instance, 5 entities, 20 synthetic clinicians, 3 visit types and their windows

The referral lands; the patient gets a text

WHAT IT DOES

- Arya watches the pre-admission queue for a new referral. The demo used a mocked HCHB intake queue, with Cornelius Ashgrove, received 27 Sep and no SOC yet.
- Its first step is to confirm the patient is actually going home. It texts the patient (“Just checking to see if you’ve been discharged yet?”). The reply “discharged late tomorrow” sets the SOC window to 8 pm Tuesday through 8 pm Wednesday.
- The same thread captures patient preferences (“I want to sleep in the afternoon... come in the morning”), which the agent notes for scheduling.

FOR COMPASSUS

- Both sides agreed Arya should take the referral from our intake platform, not from HCHB. Kunal: “We’ll want this to potentially write into HCHB as opposed to read from it.” 0:22:58
- Michael sees the welcome-call script as the place to collect preferences before any clinician calls. Colin: preferences set loosely at SOC “poison” the plan of care for every later discipline. 1:00:55 1:01:35
- Colin: a welcome call made at 1 pm instead of 4 pm “could have accepted two more referrals.” Earlier patient contact is a capacity lever. 1:22:38

“This is not another software that’s built for your office staff to log into. This is a software and agent that’s supposed to do the work.”

Kunal Sarda • 0:25:27

Ashgrove, Cornelius

MRN Assigned on accept

DOB 11/02/1946

BRANCH Pomona Home Health

PRIMARY MD Dr. Priya Vandermolen, MD

Pre-Admission

Verify Eligibility

Accept Referral

Refuse / Non-Admit

INTAKE COORDINATOR

Intake Team

Referral header

REFERRAL ID

REF-1011

RECEIVED

9/27/2026, 6:39:15 PM

SOURCE

Dr. Priya Vandermolen, MD (Physician)

CONTACT

Janelle Ortiz (909) 555-0116

ELIGIBILITY

Eligible 9/27/2026

STATUS

Pre-Admission

Patient & visits

Open Client Profile

Date	Time	Service	Clinician	Status
Waiting on the SOC visit. Visits posted to /api/visits for this patient appear here.				

Referral

REFERRAL SOURCE

Dr. Priya Vandermolen, MD

SOURCE TYPE

Physician

SERVICE LINE

Home Health

BRANCH

Pomona Home Health

REFERRAL CONTACT NAME

Janelle Ortiz

CONTACT PHONE

(909) 555-0116

REQUESTED SOC DATE

09/29/2026

Patient demographics

FIRST NAME

Cornelius

LAST NAME

Ashgrove

DOB (MM/DD/YYYY)

11/02/1946

GENDER

Male

SSN (LAST 4)

7300

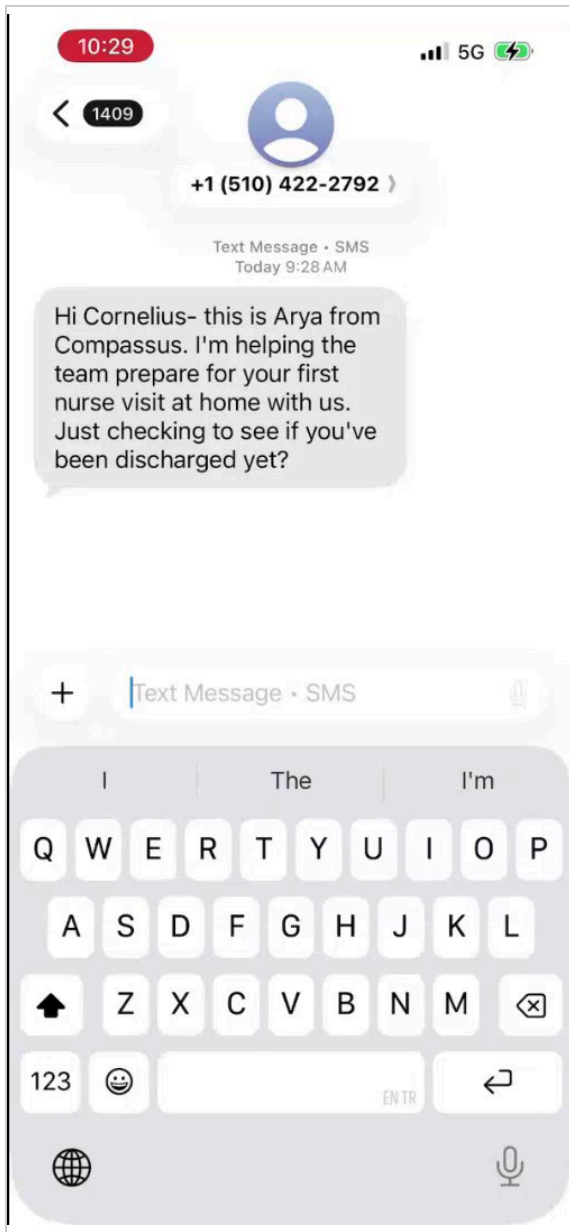
PHONE

(909) 555-0120

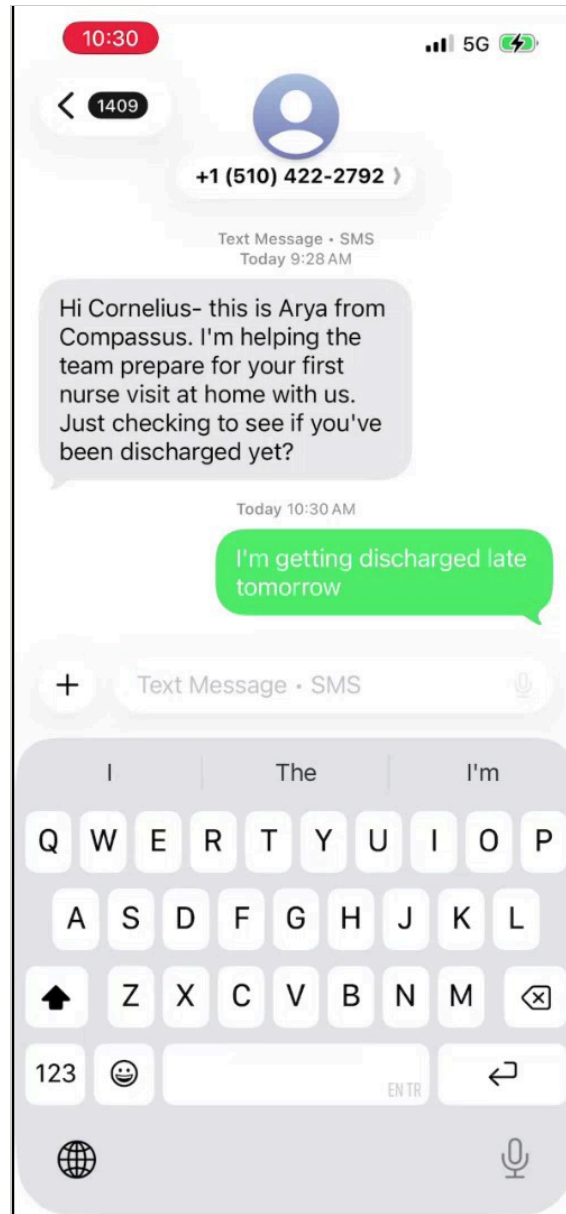
STREET

1503 W. Holt Ave.

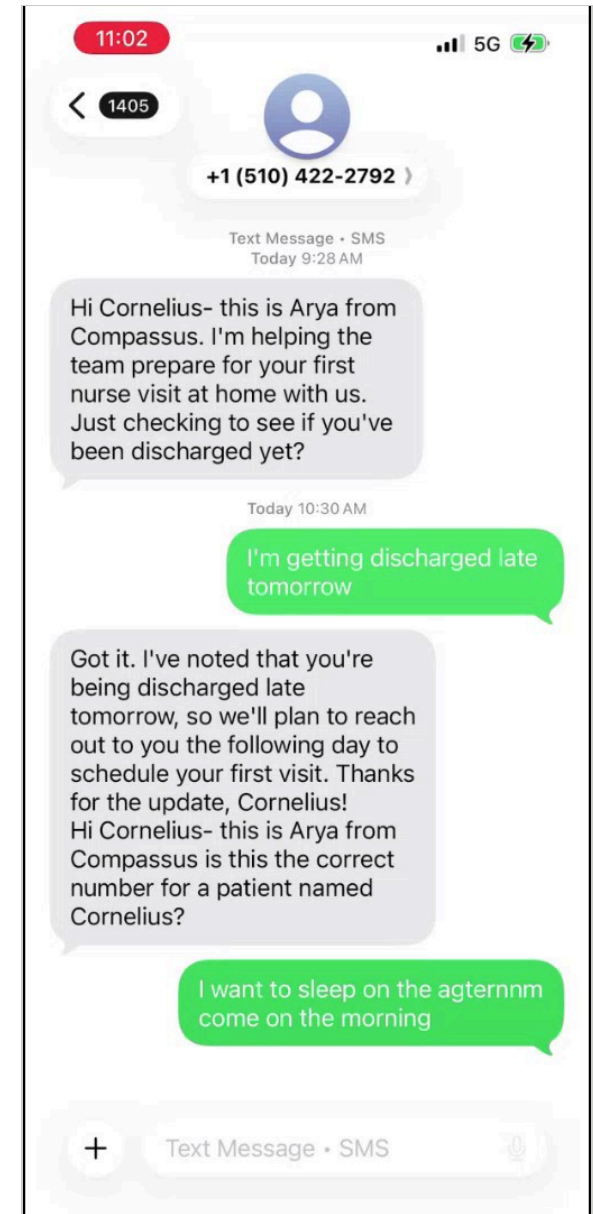
0:23:45 Cornelius Ashgrove's referral: eligible, pre-admission, no start of care scheduled yet



0:25:30 Arya texts the patient to check on discharge



0:26:21 "I'm getting discharged late tomorrow." This sets the SOC window



0:58:12 A patient preference captured in the same thread: mornings, not afternoons

Who, with a score, and when, with a reason

WHAT IT DOES

- The home screen is a worklist (“Good morning! 12 patients need your immediate attention”). Each visit shows its window, the number of recommendations, and a status of *Finding coverage* or *Coverage found*.
- Opening a patient shows eligible clinicians ranked by match: Quilla Lindqvist, 100%, 1.6 mi, 18 points, full-time; Rowan Marbury, 72%, 1.5 mi, 20.5 points, part-time.
- Each clinician comes with a recommended slot (Wed 30 Sep, 1:42-2:42 pm). Arya picks it by fitting the SOC into her existing day for the least total drive time.
- Kunal says the panel shows why the clinician is the best fit, e.g. no overtime, continuity, closest to the patient, matches preferences. 1:52:31

FOR COMPASSUS

- This is common question 1 answered at the clinician level: who could take this patient, and when. It is also the only product in the round that proposes a time, not just a person.
- Evan’s outlier question is still open. When the best overall answer ruins one clinician’s day, “this is a question... we’re still working through.” 1:31:57
- Arya says scheduling should run up to a month ahead, within the auth window, not the night before. 1:30:47 1:45:15

“The time that you saw Arya recommend, it was actually plotting the least amount of distance that someone has to travel.”

Cornelius Ashgrove

📅 Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm



🔍 Recommended

📅 All Visits

📄 Patient Details

🔍 Search by name

🏠 RN

2 ▼

Quilla Lindqvist

100% Match

📍 1.6 mi ⚡ 18 pts

Full-Time

Rowan Marbury

72% Match

📍 1.5 mi ⚡ 20.5 pts

Part-Time

🔍 Quilla Lindqvist Cleared

100% match • 1.6 mi away • 18 pts • Full-Time

📞 +17145551117

Recommended Availabilities

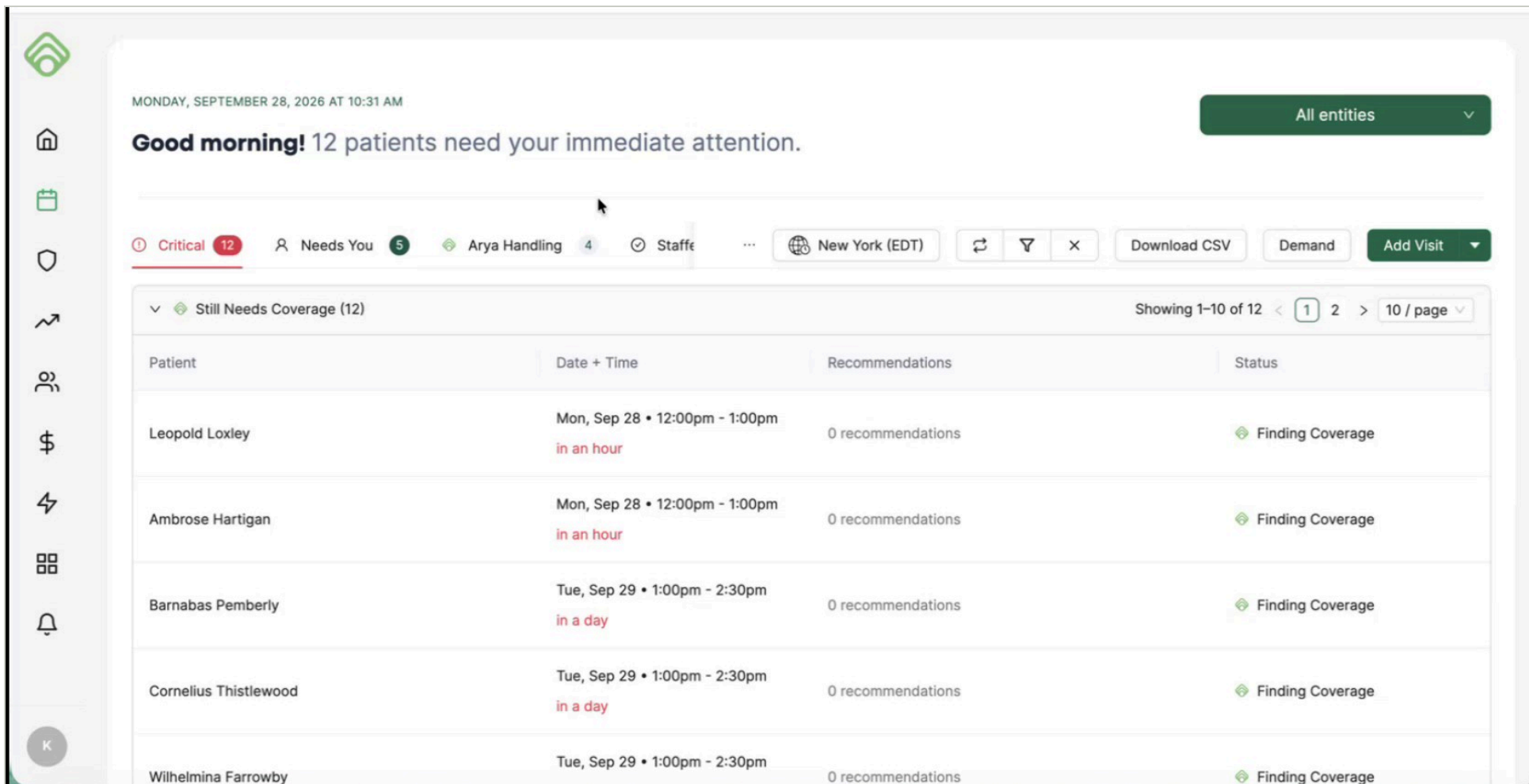
Accept this clinician for individual visits.

> Wed, Sep 30 1:42 PM - 2:42 PM m 14 hours ago

👍 Accept



0:30:33 Recommended: Quilla Lindqvist, 100% match, 1.6 mi, 18 pts, full-time, with a suggested slot of Wed 30 Sep 1:42-2:42 pm ZOOMED



0:27:30 The office worklist: every open visit with its window, number of recommendations and coverage status

The screenshot displays a medical application interface. On the left, a vertical sidebar contains icons for home, calendar, location, route, people, dollar sign, lightning bolt, grid, and bell. The main area is divided into two panels. The background panel is a worklist with patient names and visit dates/times. The foreground panel, titled 'Cornelius Ashgrove', shows visit dates 'Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm'. It has tabs for 'Recommended', 'All Visits', and 'Patient Details'. The 'Recommended' tab is active, showing a search bar and a list of recommended clinicians. A dropdown menu for 'RN' is open, showing 'Quilla Lindqvist' (100% Match, 1.6 mi, 18 pts, Full-Time) and 'Rowan Marbury' (72% Match, 1.5 mi, 20.5 pts, Part-Time). To the right of the dropdown, details for Quilla Lindqvist are shown, including a 'Cleared' status, '100% match • 1.6 mi away • 18 pts • Full-Time', and a phone number '+17145551117'. Below this, a section titled 'Recommended Availabilities' shows a date/time selector for 'Wed, Sep 30' from '1:42 PM' to '2:42 PM' (14 hours ago) with an 'Accept' button.

Patient Name	Visit Date/Time
Leopold Loxley	Mon, Sep 28 • 12:00pm in an hour
Ambrose Hartigan	Mon, Sep 28 • 12:00pm in an hour
Barnabas Pemberly	Tue, Sep 29 • 1:00pm in a day
Cornelius Thistlewood	Tue, Sep 29 • 1:00pm in a day
Wilhelmina Farrowby	Tue, Sep 29 • 1:00pm in a day
Cornelius Ashgrove	Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm in a day
Octavia Brennicke	Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm in a day
Philippa Ashdown	Wed, Sep 30 • 4:00pm in 2 days
Thaddeus Carrow	Wed, Sep 30 • 4:00pm

Cornelius Ashgrove
Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm

Recommended | All Visits | Patient Details

Search by name

RN 2

Quilla Lindqvist
100% Match
1.6 mi 18 pts
Full-Time

Rowan Marbury
72% Match
1.5 mi 20.5 pts
Part-Time

Quilla Lindqvist **Cleared**
100% match • 1.6 mi away • 18 pts • Full-Time
+17145551117

Recommended Availabilities
Accept this clinician for individual visits.

> Wed, Sep 30 1:42 PM - 2:42 PM 14 hours ago **Accept**

0:30:33 The full screen: the patient panel opens over the worklist

Office approves; the clinician confirms by text

WHAT IT DOES

- In co-pilot mode the office accepts, rejects or puts a recommendation in review. Alerts can go by Teams, text or email.
- A demo rule: accept a **full-time** clinician and Arya writes the SOC to HCHB at once. Accept a **part-time/PRN** clinician and it waits: Arya texts them a link to approve the patient first.
- Other setups: Arya texts or calls eligible clinicians in batches, best fit first (“not blast everyone and their uncle”). Clinicians apply, and the office sees applicants with their coverage (e.g. “Fenwick Delacorte, 88.5%, 15 pts, part-time”).
- Later the office moves from *approving* to being *informed*. It sees only the critical items: “No clinicians seem to be available. I need you to figure out if anyone can take this case.”

“Is the office staff in the loop versus the office staff approving things.”

Kunal Sarda · 1:25:27

FOR COMPASSUS

- Michael asked what the scheduler does and whether the clinician keeps control. The answer: nothing lands on a clinician’s schedule without their yes, unless we set full-time assignments to be automatic. 1:27:49
- Colin: Arya would feel like “another teammate on the team.” Thursday-afternoon coverage for three evals could be batched to all PTs automatically. 1:28:15 1:29:24
- The approval step is presented as a phase for building trust. Our Assist boundary needs it available permanently, per action.

Cornelius Ashgrove

📅 Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm



🔍 Recommended

📅 All Visits

📄 Patient Details

🔍 Search by name

🏠 RN

2 ▼

Quilla Lindqvist

100% Match

📍 1.6 mi ⚡ 18 pts

Full-Time

Rowan Marbury

72% Match

📍 1.5 mi ⚡ 20.5 pts

Part-Time

🔍 Rowan Marbury Cleared

72% match • 1.5 mi away • 20.5 pts • Part-Time

📞 +15402502633

Recommended Availabilities

Accept this clinician for individual visits.

> Wed, Sep 2:50 - 3:50 PM rn 14 hours ago Awaiting Acknowledgement

0:31:51 Rowan Marbury, part-time, 72%: accepted by the office and now "awaiting acknowledgement" from the clinician ZOOMED

Everett Quillon

📅 Thu, Oct 1 • 8:00pm - Fri, Oct 2 • 8:00pm



🔍 Recommended

📅 All Visits

📋 Patient Details

🔍 Search by name

🏠 RN

1 ▼

Fenwick Delacorte

88.5% Match

📍 1.6 mi ⚡ 15 pts

Part-Time

🔍 Fenwick Delacorte Cleared

88.5% match • 1.6 mi away • 15 pts • Part-Time

📞 +17145551106

Recommended Availabilities

Accept this clinician for individual visits.

> Fri, Oct 2 3:00 PM - 4:00 PM rn 14 hours ago

✓ Accept



1:04:54 Applications to review: Fenwick Delacorte, 88.5% match, 1.6 mi, 15 pts, part-time, for Fri 2 Oct 3-4 pm ZOOMED

The SOC appears in HCHB, closing the loop

WHAT IT DOES

- Once the clinician is confirmed, Arya plots the SOC in HCHB: date, time, service and clinician (Quilla Lindqvist), with the status Scheduled.
- In production this runs through Citrix screen automation: reads every 15-20 minutes and writes about every 15 minutes, depending on logins provisioned and data volume.
- Single sign-on such as Okta can log the bot out every 20-30 minutes, which slows every cycle. Arya will load-test our sandbox against the agreed times.
- Sources of truth stay where they are: HCHB for visits, frequencies and auth windows; Workday for clinician demographics and status; our intake platform for its data.

FOR COMPASSUS

- Evan asked if anything still has to happen in HCHB. Anand: “Preferably... nobody has to use HomeCare HomeBase for day-to-day tasks,” depending on what we keep there. **1:48:58**
- 15-20 minutes is well short of the under-five-minute latency in their return, and in the range that made Axle switch its integration off. Ask for measured numbers from the sandbox.
- Screen automation breaks when HCHB changes a screen. Anand: “each customer... the way some things are set up is slightly different.” Detection and repair SLAs were not discussed.

“I think their marketplace and their API are supposedly coming in 2027... Citrix-based browser automation, the usual trio.”

No app: a text thread and short links

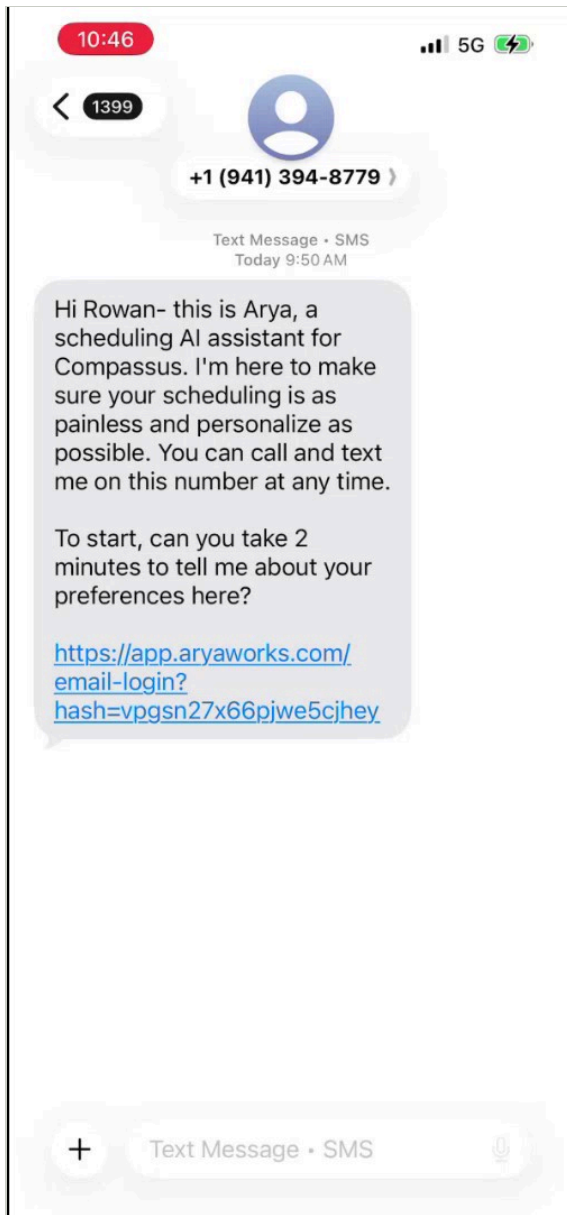
WHAT IT DOES

- When Arya sees a new clinician in HCHB, it texts them an introduction and a link to set their preferences. Login is by a code sent by text, with no password.
- The preferences are the days they like to work, the times of day (early morning to overnight), and the regions they will cover, chosen by town under each branch. Arya re-checks them proactively every couple of weeks.
- “What’s my schedule tomorrow?” returns a link to the day’s route on a map, with drive time and patient time, an adjustable start time, and Navigate and Share buttons.
- On save, Arya can tell patients when to expect the clinician. The clinician can also send it from their own phone (“I expect to get to you at around 9:41 AM”).

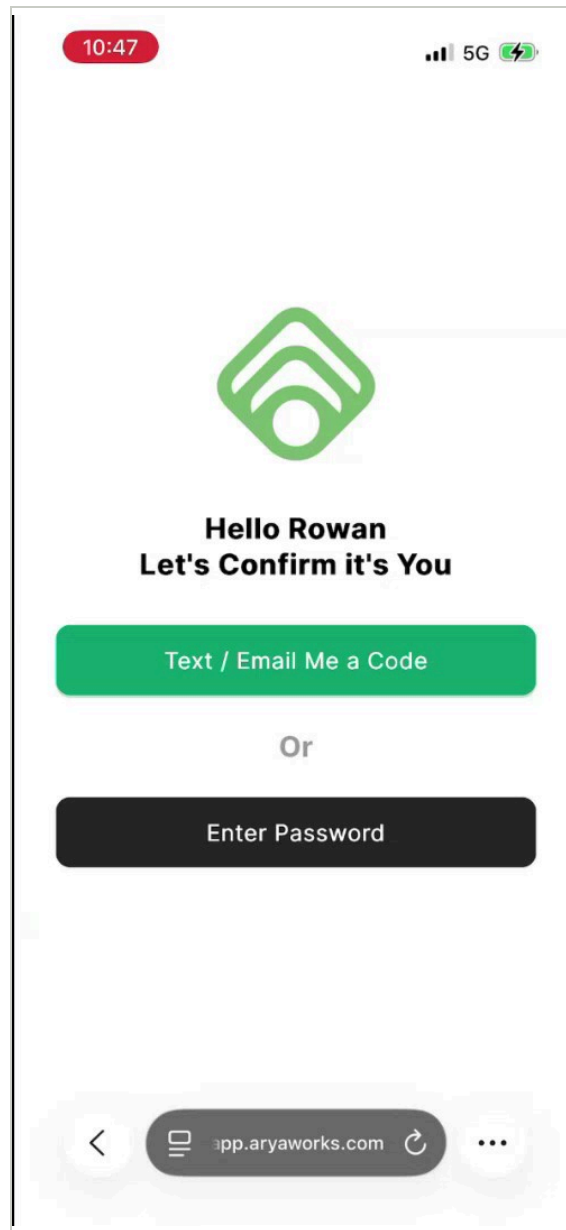
FOR COMPASSUS

- Colin: onboarding, alongside the clinical manager, is the right moment to set territory expectations, “instead of having a clinician fill that out.” 0:44:50
- Evan wants patient messages to come from a Compassus number. Arya can use our telephony (e.g. Twilio) if we have one. 0:53:39 0:54:35
- Older clinicians prefer to call and younger ones to text, so Arya offers both. How often and when Arya may contact each clinician is a captured preference. 1:04:51

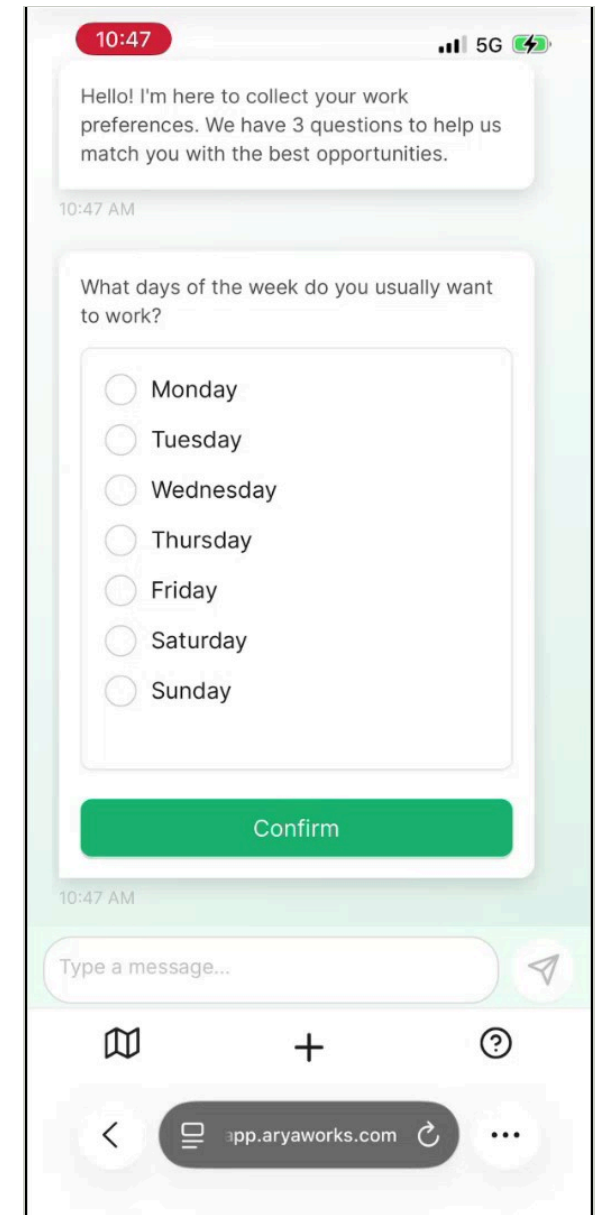
“We have a very strict never-build-an-app policy for clinicians, because no clinician wants to have yet another app that they need to log into.”



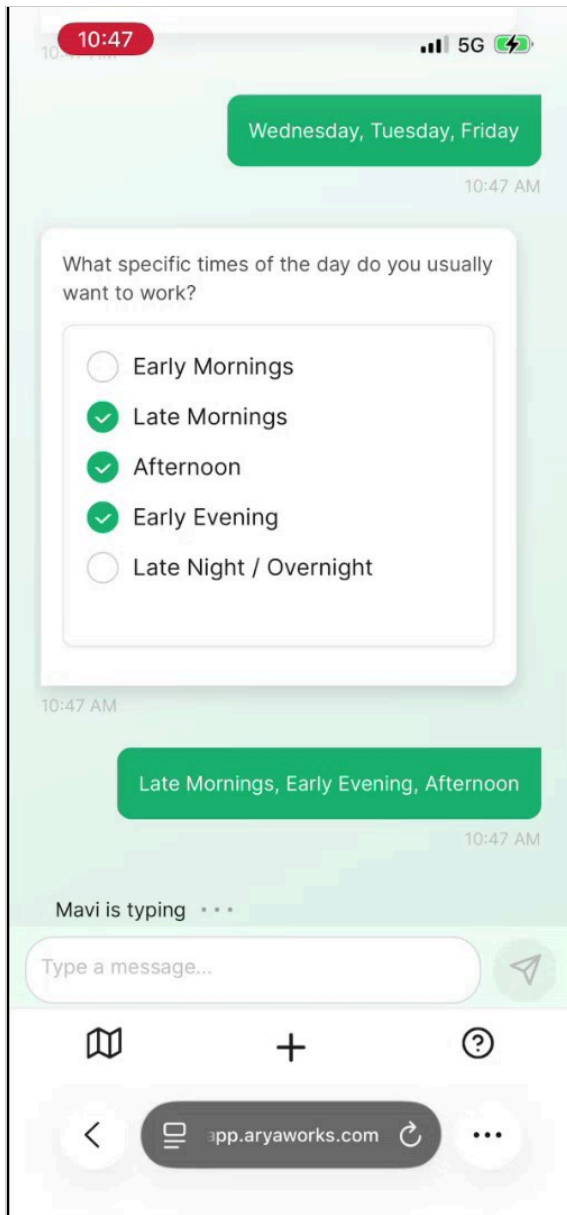
0:42:30 The first text to a new clinician, with a link to set preferences



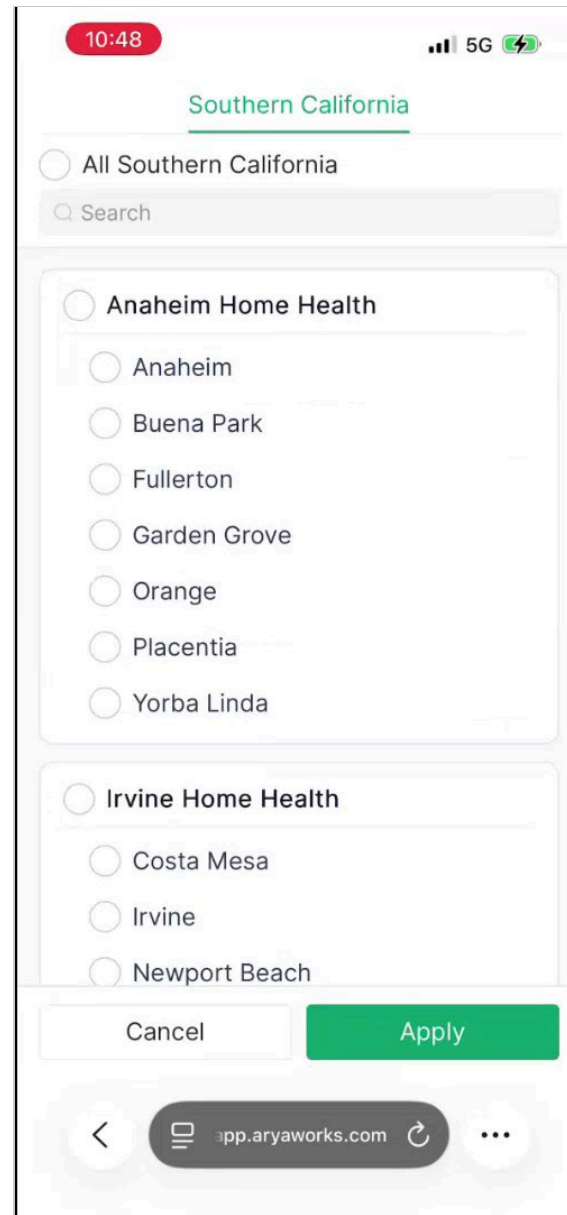
0:43:08 No password: a code by text or email



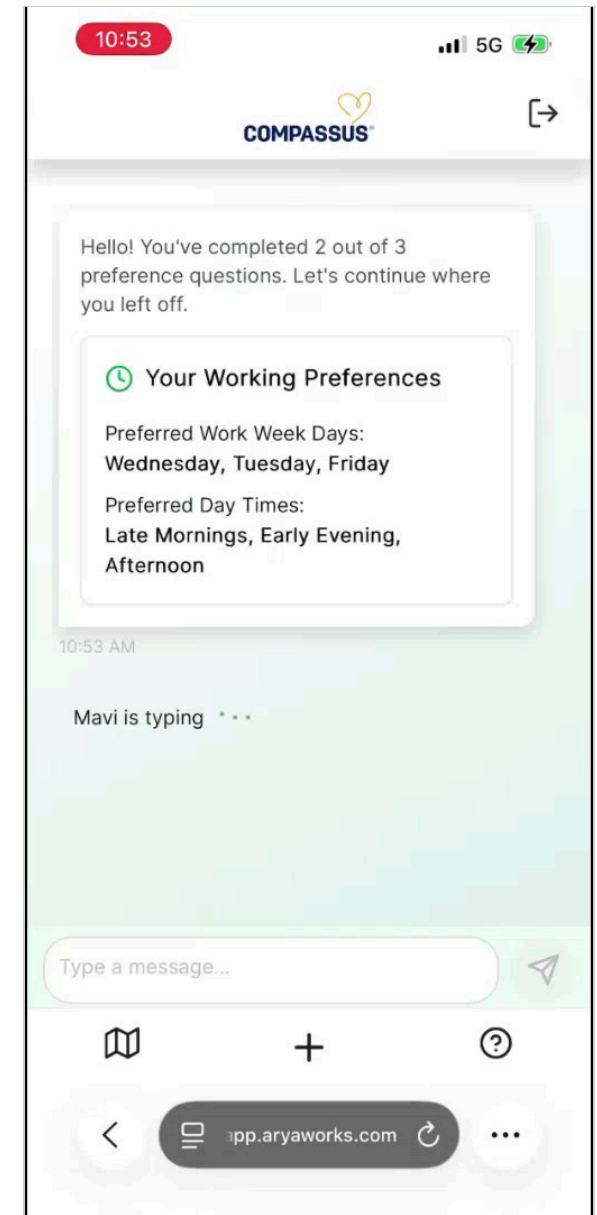
0:43:24 Question 1: which days do you usually want to work?



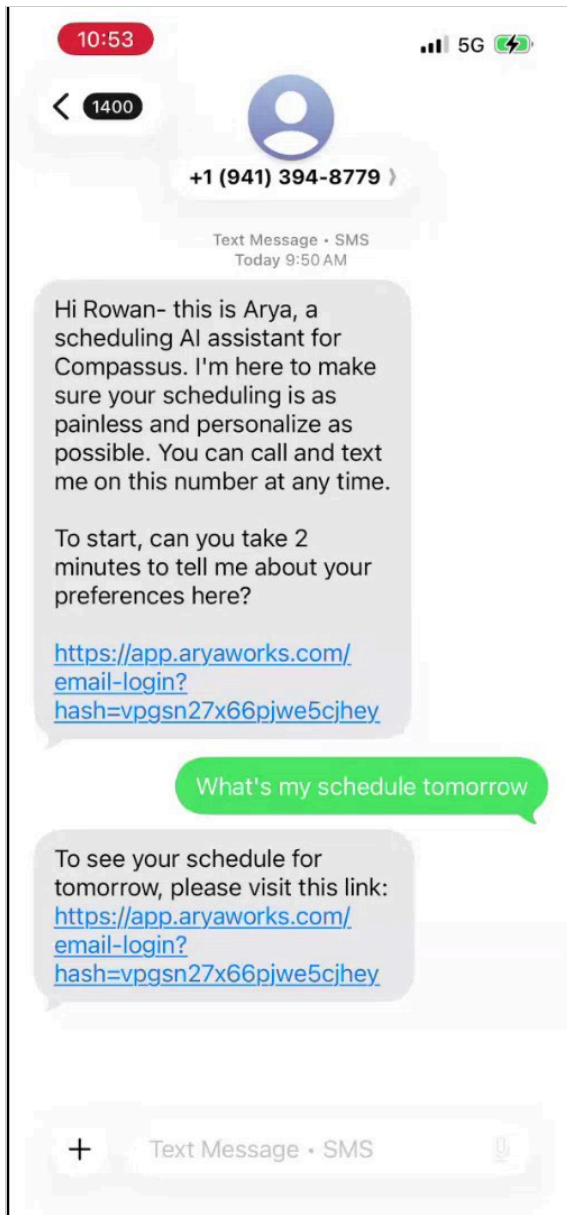
0:43:42 Question 2: which times of day? The answers come back into the chat



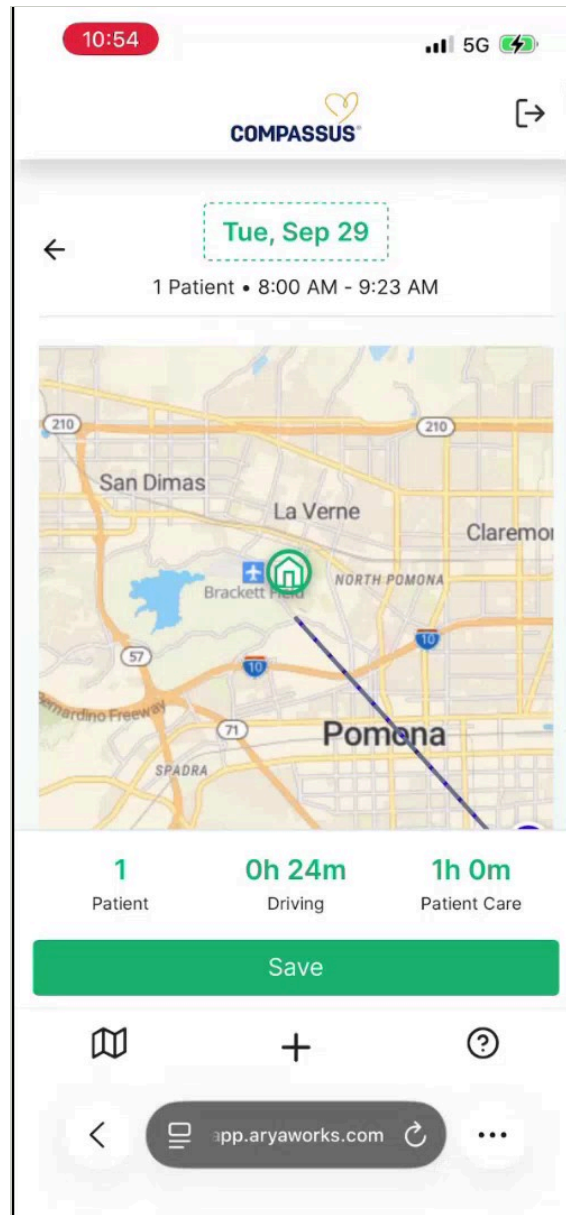
0:43:51 Question 3: which regions? Towns grouped by branch (Anaheim, Irvine)



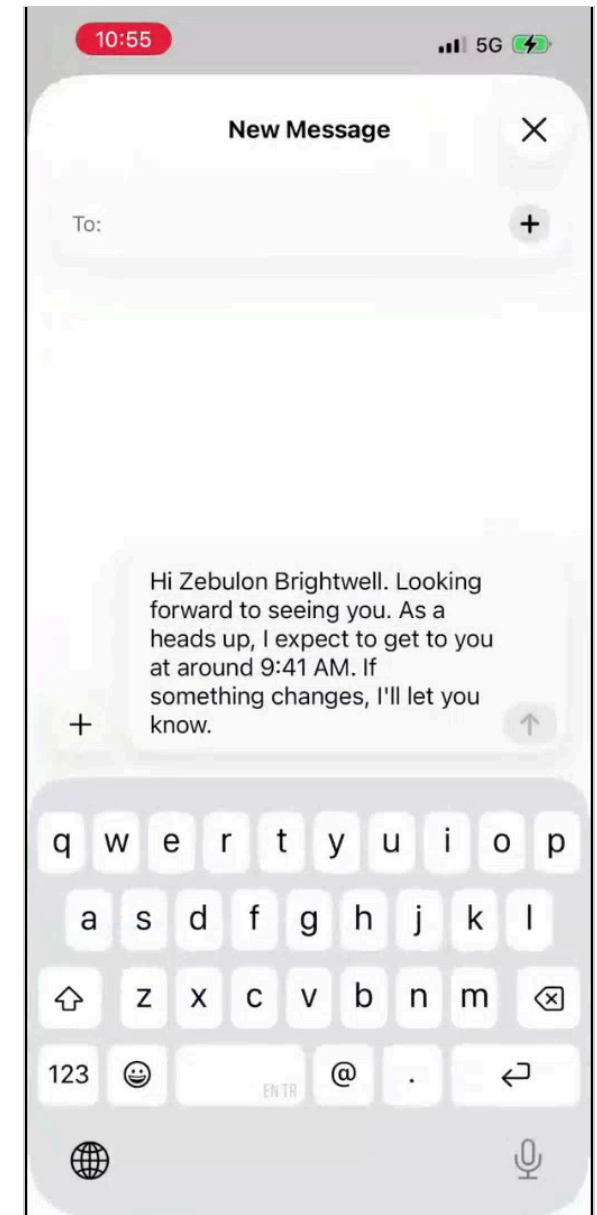
0:49:33 Saved working preferences: Wed, Tue, Fri; late mornings, early evening, afternoon



0:49:12 "What's my schedule tomorrow?" returns a short link



0:50:30 Tomorrow's route: 1 patient, 0h 24m driving, 1h patient care, adjustable start



0:51:12 Share: a ready-made heads-up to the patient from the clinician's own phone

A call-out is one text; the agent re-forms the week

WHAT IT DOES

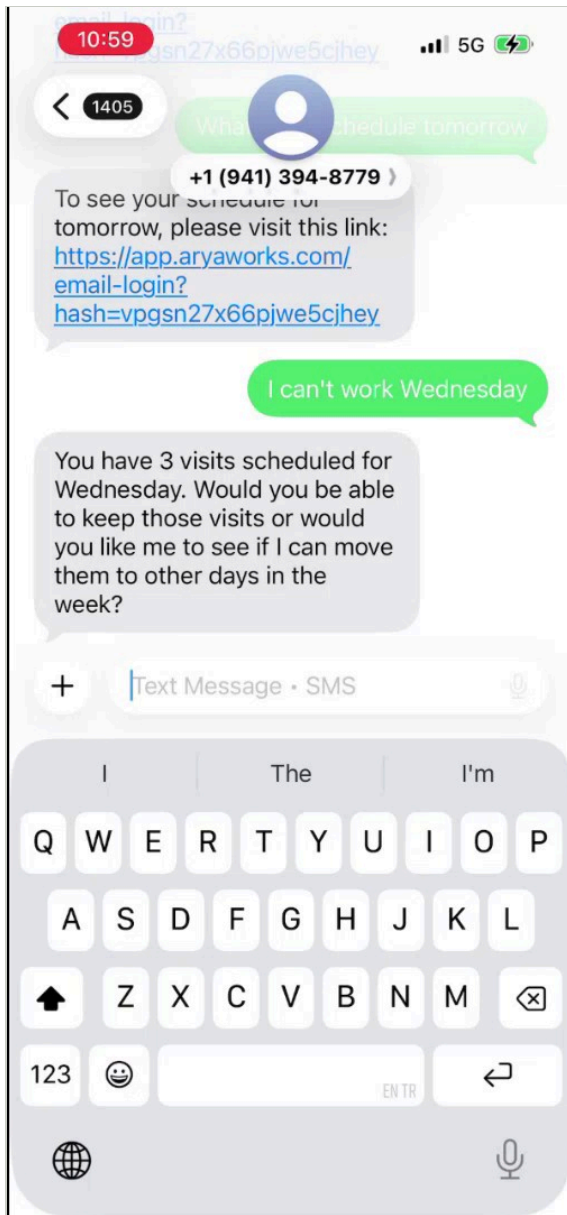
- The clinician texts “I can’t work Wednesday.” Arya lists the three visits and asks which to keep or move. The reply is “I can’t work any of these.”
- Arya first tries to move the visits within the same clinician’s week, inside each visit’s capacity window. If it can’t, it raises call-outs, re-staffs them, and tells the office.
- Applicants come with Arya’s own recommendation, e.g. “Tami Stephenson covers 3 of 10 visits with a 0% match”, and with reasons a clinician isn’t eligible (“Not eligible for 1 of 1 visit”).
- For continuity it can favour the clinician who covers most of the episode: “Antoinette Delice covers 8 of 33 visits with a 47.1% match.” Visits past the authorization window aren’t scheduled; Arya escalates, and *Put in review* holds the slot while an expected auth comes through.

“It’s not going to schedule beyond the auth. It’s going to escalate.”

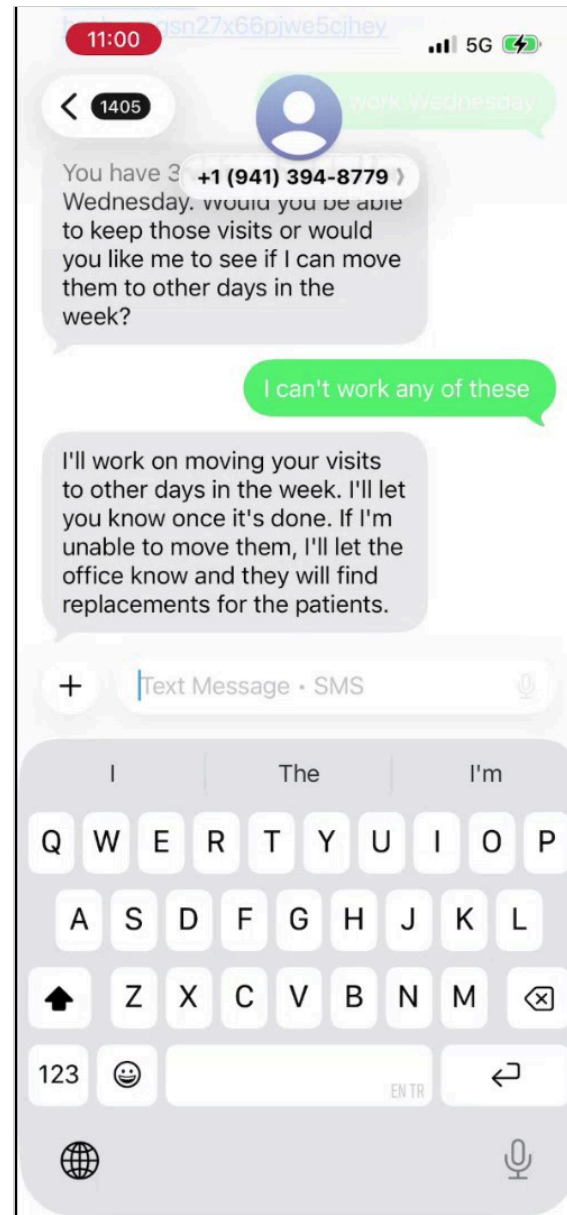
Kunal Sarda • 1:46:36

FOR COMPASSUS

- This covers most of common question 6. We did not see our full Tuesday: two 7 am call-outs plus a visit at the edge of its window.
- Colin: expected authorizations must keep a place in the plan so they aren’t forgotten. The “Put in review” hold does that. 1:46:59 1:47:42
- Hard-to-fill visits: Arya has a “negotiation scale” to offer bonuses or extra productivity points. Colin: “car washes go a long way.” 1:33:41 1:34:40
- The applicant data shown was “data out in the wild,” with low match scores throughout. 1:40:47



0:55:30 "I can't work Wednesday." Arya: "You have 3 visits scheduled... would you like me to move them?"



0:56:03 "I'll work on moving your visits to other days... if unable, I'll let the office know and they will find replacements"

Finn Price

Mon, Sep 28 • 9:00am - 9:00pm

Archive



My Recommendation

Tami Stephenson covers 3 of 10 visits with a 0% match. Preferred over Clarissa Bonilla (0% match).

Applicants

Eligible Clinicians (114)

All Visits

Patient Details

Search by name

NEW

2

Clarissa Bonilla

0% Match

37.2 mi 1 shift

Not eligible for 1 of 1 shift

Tami Stephenson

Clarissa Bonilla

New to patient

Cleared

0% match • 37.2 mi away

No overtime

+18179033182

Not eligible for 1 of 1 visit

Visits Applied For

1:39:21 Applicants for Finn Price: Arya's recommendation, 114 eligible clinicians, and the reason Clarissa Bonilla is not eligible ZOOMED

Frederick Widjaja

Mon, Sep 28 • 5:30pm - Tue, Sep 29 • 12:00am

Archive



My Recommendation

Antoinette Delice covers 8 of 33 visits with a 47.1% match.

Applicants

Eligible Clinicians (118)

All Visits

Patient Details

Search by name

NEW

1

Antoinette Delice

47.1% Match

15.8 mi 8 shifts

Antoinette Delice

New to patient

Cleared

47.1% match • 15.8 mi away

No overtime

+19452531068

Visits Applied For

Accept this clinician for individual visits or use "Accept All".

Accept All (8)



CRITICAL · WITHIN 3 DAYS (2)

> Tue, 5:30 PM - Wed, 12:00 AM Applied 2 days ago

> Wed, 5:30 PM - Thu, 12:00 AM Applied 2 days ago

Accept



Put in Review

Reject

UPCOMING VISITS (6)

1:48:00 Continuity-first recommendation (8 of 33 visits), critical visits within 3 days, and the Accept / Put in Review / Reject menu ZOOMED

Every “no” is data; every rollout needs champions

WHAT IT DOES

- **Rejection with a reason.** The office rejects a recommendation and types why (“Quilla doesn’t go to the city at all,” typos included). Arya decides whether this is a clinician, patient or organization preference and applies it next time.
- **Insights.** Shifts filled, hours staffed, caregivers engaged, open shifts. Applications by status, actions by staff member, and a rejection breakdown (“Why staff overrode Arya”).
- **Agent health.** An outreach funnel (21 eligible, 6 contacted, 6 applied, 1 approved) and eligible clinicians per shift. The funnel shows whether the rules are too tight to staff anyone.
- **Change management.** A branded “Welcome to Arya” training hub with “Ask me anything,” and a Compassus-branded rewards program: gift cards for day-one staffing and weekly challenges.

FOR COMPASSUS

- Colin asked how to see and clean up the rules Arya has learned. The answer: “we’re building the visualization... we’ve actually just figured out how to visualize that.” Not shown. 1:51:28 1:51:58
- Evan’s concern: if one clinician can teach the agent a rule, the tenured-clinician “gamesmanship” we see today could be automated.
- Their playbook (forward-leaning branch leaders, internal case studies, clinician advocates, 90-day rewards) mirrors ours. Evan: “you guys think about it exactly the same way.” 1:35:08 1:38:47

“The number one thing that’s going to make clinicians believe they should do this is other clinicians.”

Kunal Sarda · 1:35:54

Octavia Brennicke

📅 Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm



🔍 Recommended

📅 All Visits

📄 Patient Details

🔍 Search by name

🏠 RN

2 ▼

Quilla Lindqvist

100% Match

📍 1.0 mi ⚡ 18 pts

Full-Time

Rowan Marbury

75.9% Match

📍 1.4 mi ⚡ 20.5 pts

Part-Time

🔍 Quilla Lindqvist Cleared

100% match • 1 mi away • 18 pts • Full-Time

📞 +17145551117

Help us improve matching — why are you rejecting this application?

Recomm

Quilla doesn't go to this ity at all.

Accept t

> Wed, 1:42 - 2:42 PM 14 hours ago
Sep 30 PM

Cancel

Confirm Rejection

0:35:21 "Help us improve matching: why are you rejecting this application?" The free-text reason becomes a rule ZOOMED

K

Leopold Loxley

Mon, Sep 28 • 12:00

in an hour

Ambrose Hartigan

Mon, Sep 28 • 12:00

in an hour

Barnabas Pemberly

Tue, Sep 29 • 1:00pm

in a day

Cornelius Thistlewood

Tue, Sep 29 • 1:00pm

in a day

Wilhelmina Farrowby

Tue, Sep 29 • 1:00pm

in a day

Octavia Brennicke

Tue, Sep 29 • 8:00pm - 30 • 8:00pm

in a day

Thaddeus Carrow

Wed, Sep 30 • 4:00pm

in 2 days

Philippa Ashdown

Wed, Sep 30 • 4:00pm

in 2 days

Evelyn Marchetti

Wed, Sep 30 • 8:00pm - 8:00pm

Octavia Brennicke

Tue, Sep 29 • 8:00pm - Wed, Sep 30 • 8:00pm

Recommended

All Visits

Patient Details

Search by name

RN

2

Quilla Lindqvist

100% Match

1.0 mi 18 pts

Full-Time

Rowan Marbury

75.9% Match

1.4 mi 20.5 pts

Part-Time

Quilla Lindqvist

Cleared

100% match • 1 mi away • 18 pts • Full-Time

+17145551117

Recommended Availabilities

Accept this clinician for individual visits.

> Wed, Sep 30

1:42 PM - 2:42 PM

m

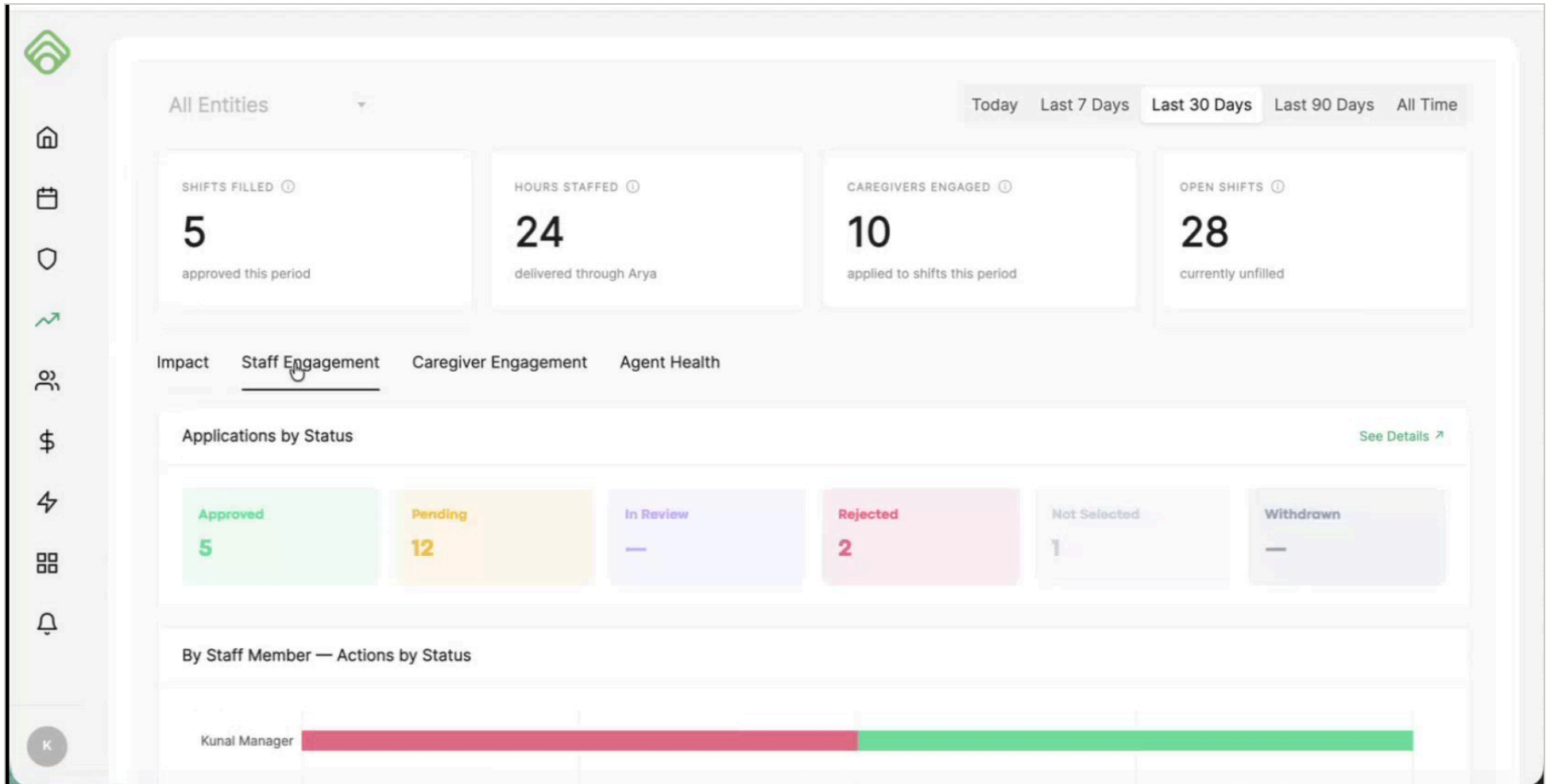
14 hours ago

Accept

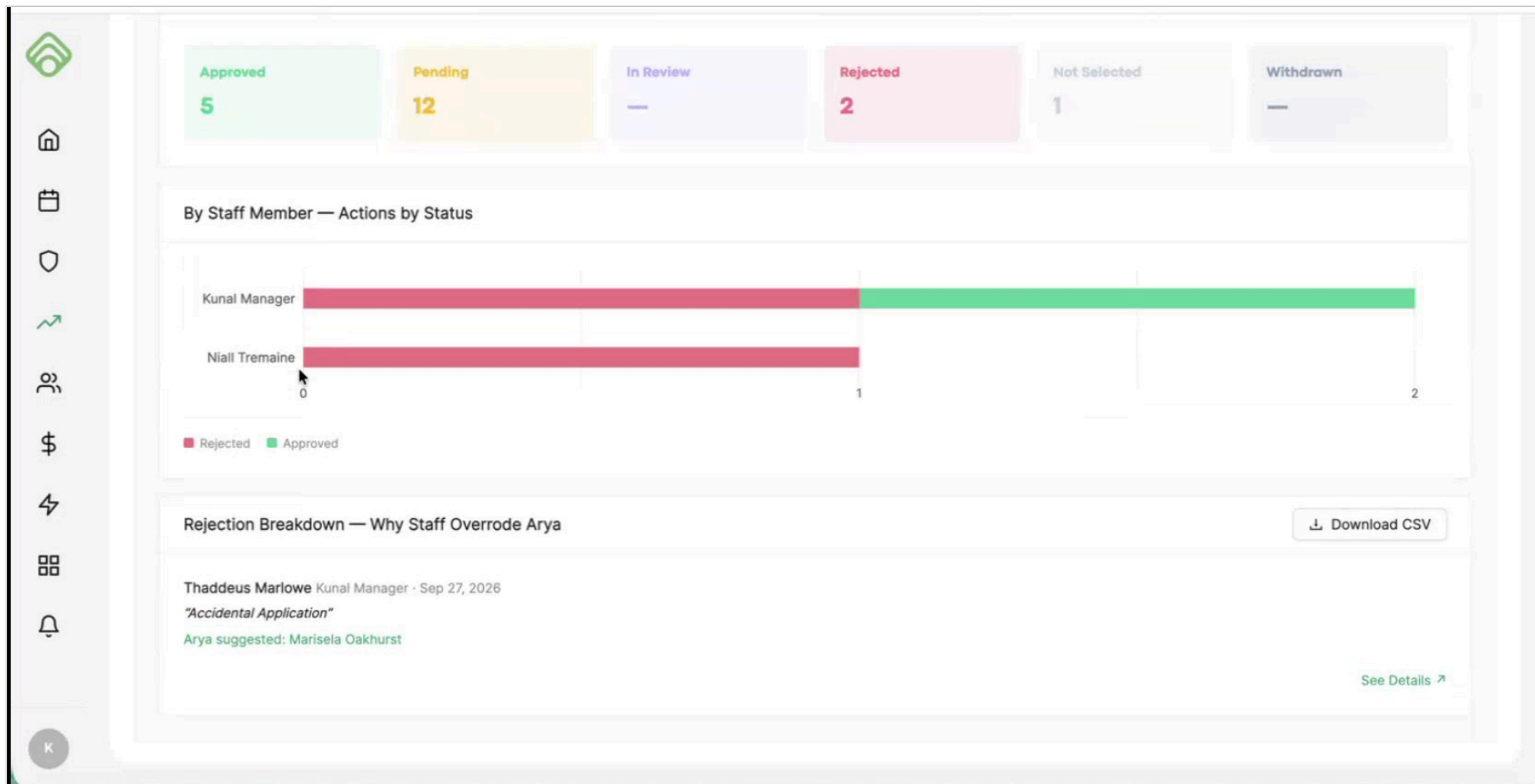
Put in Review

Reject

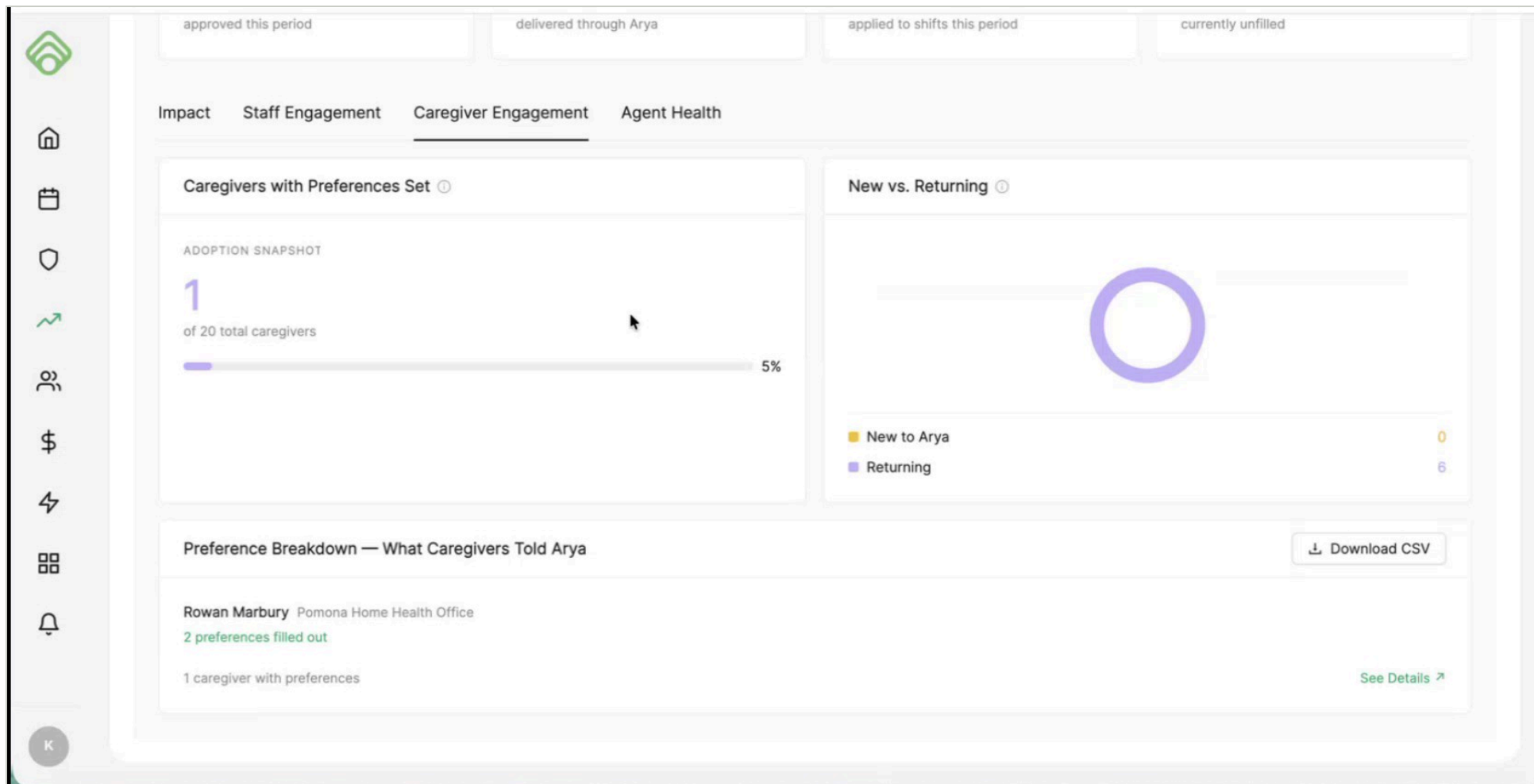
0:34:57 The office's three options on any recommendation: Accept, Put in Review, Reject



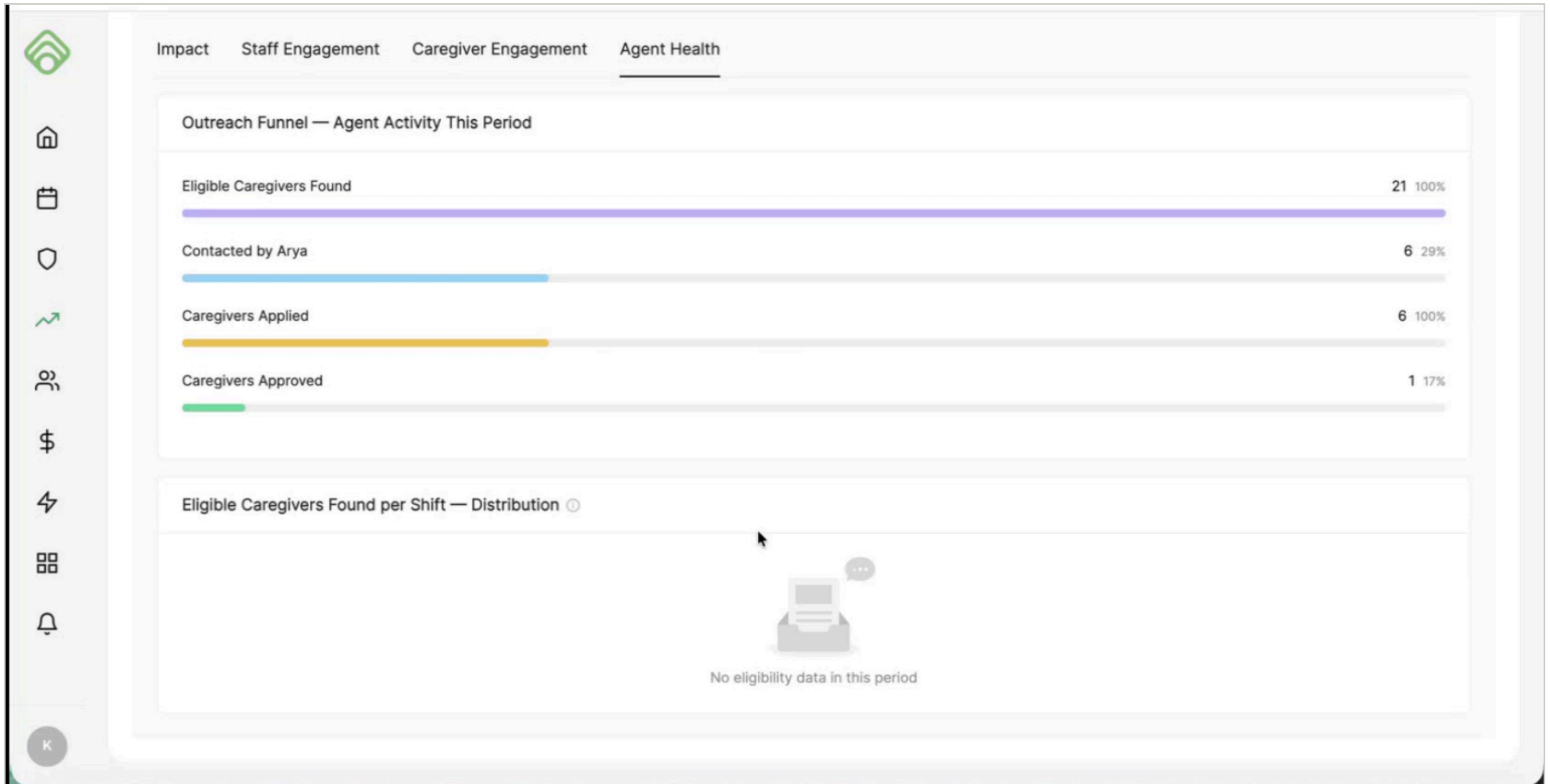
1:17:21 Insights: shifts filled, hours staffed, caregivers engaged and open shifts, by entity and period



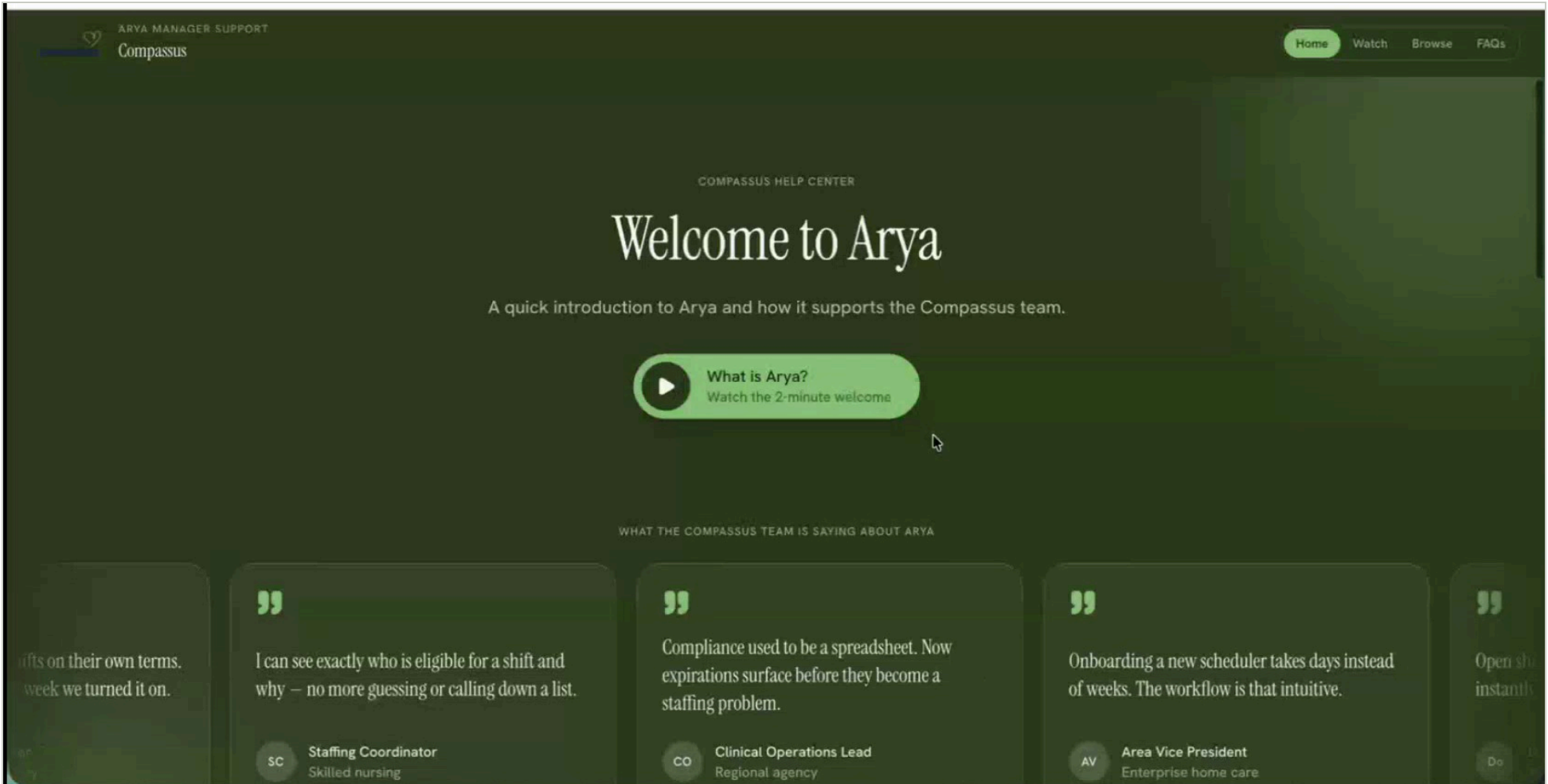
1:17:51 Staff engagement: applications by status, actions by staff member, and "Rejection breakdown: why staff overrode Arya"



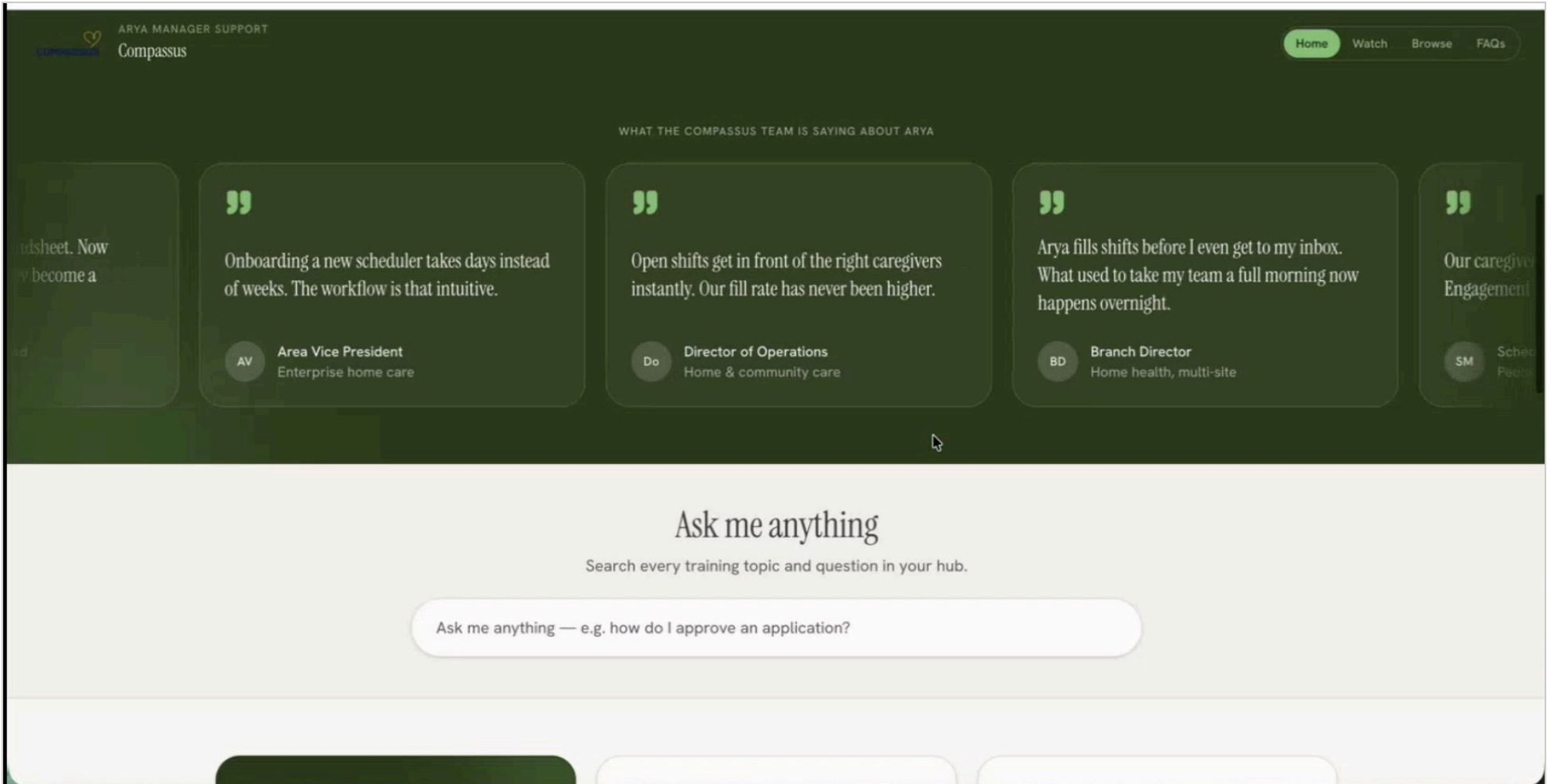
1:18:24 Caregiver engagement: who has set preferences, new vs returning, and what caregivers told Arya



1:19:12 Agent health: the outreach funnel from eligible to contacted, applied and approved



1:36:12 "Welcome to Arya": a training hub for the Compassus team, with staff testimonials



1:36:31 "Ask me anything": search every training topic and question in the hub



Rewards Program

Compassus

[← Back to Dashboard](#)

Earn rewards for your achievements

Your Path to Rewards

Complete challenges, hit milestones, and earn exciting rewards as you master new tools and drive success.

Day 1 Rewards



\$10 gift card

Early Bird

Anyone who staffs a visit on Day 1.

HOW TO EARN

Staff a visit using Arya on launch day.

Week 1



\$30 Gift Card

Sprinter

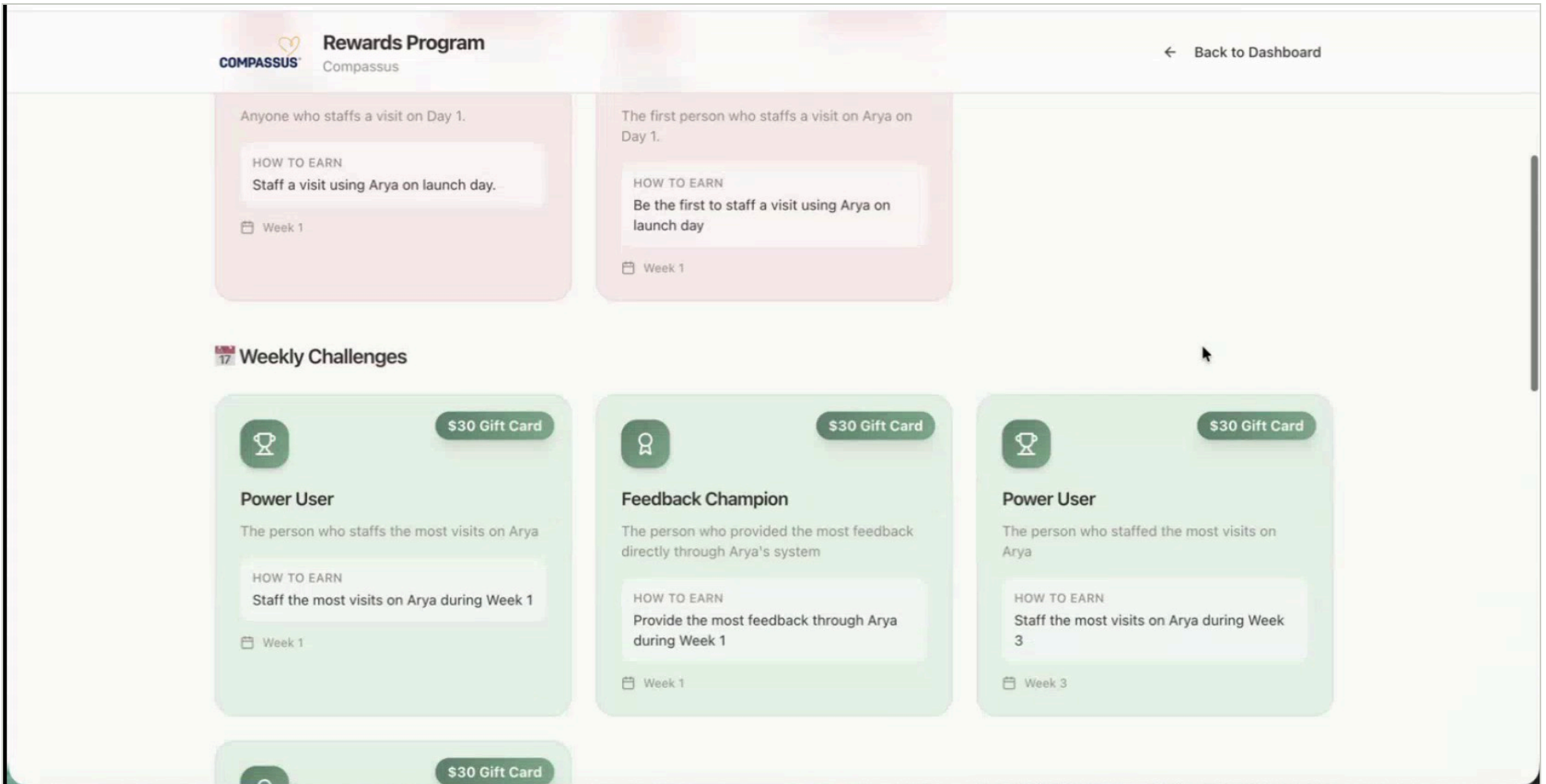
The first person who staffs a visit on Arya on Day 1.

HOW TO EARN

Be the first to staff a visit using Arya on launch day

Week 1

1:36:51 A Compassus-branded rewards program: day-one rewards for the first visits staffed through Arya



1:37:21 Weekly challenges: \$30 gift cards for Power User and Feedback Champion

Impact and positioning

CLAIM	AS STATED	OUR READ
Clinician call time	● "CLOSE TO ZERO" The clinician handles exceptions only, in place of 30-60 min a day of patient calls 1:12:13	The biggest claim of the call, and no customer or baseline was given. It is our business case, so it needs a pilot measure.
HCHB latency	● 15-20 MIN READS About 15 min writes, subject to SSO logouts; load-tested on our sandbox 1:53:37	More candid than the return's "under five minutes." Get sandbox numbers at our read and write volume before the on-site.
Engagement	Text and short links give "higher engagement" than an app 0:50:22	Plausible for our clinicians. No rate given; ask for response rates by channel.
Incentives	Bonuses and extra points for hard-to-fill visits "work" 1:34:17	Anecdotal. The only incentive mechanism shown in the round beyond a manual waitlist.
Scale	"Organizations with hundreds of locations, hundreds of branches"; Bayada, Pennant and Aveanna logos 0:01:41	Enterprise logos, but no HCHB customer count and no production dates for HCHB.
Capacity model	RFP return: capacity kept per clinician in the customer's own unit (points, visits or hours), aggregated by branch; each referral checked for SOC-eligible room and episode impact; blocked visits held until auth or consent clears (C1, C3, C4)	The strongest capacity design in any return, and the part of the demo that was skipped. Ask for it on real data before the on-site. Scorecard: capacity 92%.
Headline result	Aveanna, 11% more authorized hours delivered (return)	Not discussed. We still don't know if it came from home health or private duty.
Breadth	One agent with intake, talent (recruiting), front-desk voice and scheduling skills sharing a "common context layer" 1:02:23	An answer to our upstream data problem, and a broader dependency on one vendor. Our intake platform already covers intake.

Question by question

● ANSWERED 3 ● PARTLY 8 ● NOT ANSWERED 1 ● NOT ASKED 5

QUESTION	STATUS	WHAT WE HEARD	WHEN
Common 01 The referral decision	● PARTLY	Shown: SOC staffing from pickup to write-back. Not shown: the accept/decline view (deferred for lack of good data). RFP return C1: “whether there are clinicians who are clinically and operationally eligible to perform the SOC within the required window... and how accepting the referral affects capacity for the subsequent visits.” No forecast.	1:22:17 • 1:47:33
Common 02 The episode, not the visit	● PARTLY	Capacity windows (24 h SOC, Medicare week, fixed-day wound); frequencies booked ahead within the auth; escalation before the auth runs out; review holds. RFP return C4: keeps flexible visits movable to protect capacity for fixed ones. LUPA, recert and OASIS appear in neither the call nor the return.	1:30:47 • 1:45:53
Common 03 HCHB freshness, with a number	● ANSWERED	Citrix browser automation: 15–20 min reads, about 15 min writes, SSO logouts a risk, sandbox load test. HCHB API expected 2027. Slower than their return claimed.	1:53:10
Common 04 Advise versus decide	● PARTLY	Outreach and call-out recovery run without per-event approval. New-case staffing: co-pilot approval to start, then the office is “in the loop” rather than approving. No customer named who moved the line.	1:25:12

QUESTION	STATUS	WHAT WE HEARD	WHEN
Common 05 The clinician says no	● ANSWERED	Shown live: reject with a free-text reason. Arya classifies it (clinician, patient or organization preference) and it becomes a rule or a weighting. Rejections are reported by reason and by staff member.	0:34:51 · 1:17:49
Common 06 A Tuesday that goes wrong	● PARTLY	A call-out by text, rescheduled within the week first, then re-staffed with the office told. Batched outreach down the ranked list. Our two-call-out, edge-of-window case wasn't run.	0:55:17 · 1:28:39
Common 07 What you don't do	● NOT ASKED	Gaps they named anyway: rules visualization in progress, capacity UI data, outbound voice not set up for the demo, the outlier trade-off.	—
Common 08 What breaks at our size	● PARTLY	"Hundreds of branches" at existing customers; SSO logouts and HCHB setup differences named as integration risks. Nothing on what broke.	0:01:41 · 1:53:37
Common 09 The one number you'd stake it on	● NOT ANSWERED	Evan asked how much clinician call time other deployments removed. The answer was "close to zero" with no customer, baseline or period.	1:09:25 · 1:12:13
Common 10 Where you'll look worse	● NOT ASKED	Unprompted, Kunal admitted the trade-off between optimizing everyone and upsetting one person is "a question... we're still working through."	1:31:57
Arya 01 HCHB customers in production	● NOT ASKED	Not covered. Anand spoke as the HCHB expert but gave no count or dates.	—
Arya 02 When HCHB changes a screen	● PARTLY	Each HCHB setup "is slightly different," so they check against our sandbox first. No detection or repair SLA discussed.	1:53:37

QUESTION	STATUS	WHAT WE HEARD	WHEN
Arya 03 Approval, permanently	● PARTLY	Approval is a configurable “hard stop.” It was framed as a way to build trust, with the office later informed rather than approving. Not confirmed as permanent per action.	1:25:12 · 1:39:10
Arya 04 No apps: a clinician’s real week	● ANSWERED	Shown: onboarding text, preferences, next-day route, patient heads-up, call-out, voice line. For clinicians who don’t reply, the patient is told the time anyway.	0:42:18 · 0:52:14
Arya 05 Before-the-visit automation: sequence and date	● PARTLY	Discharge check and patient preferences shown live. Kunal urged us to decide early how far AI goes into the patient experience; providers usually add it last. No date.	1:08:24
Arya 06 Aveanna: home health or private duty?	● NOT ASKED	Aveanna appeared on the opening slide only.	—
Arya 07 Customer mix, size, funding	● NOT ASKED	Team introduced (CEO, principal AI architect, CTO, VP Product, deployment and success leads); size and funding not given.	0:02:01

What to weigh

HIGH

Screen-automation integration

Citrix RPA at 15–20 minutes, SSO logouts, and HCHB setups that differ between customers. An agent that acts on a 20-minute-old schedule can double-book. It must prove itself on our sandbox at our volume.

HIGH

Capacity view not shown

Branch-level supply against expected demand, at the moment of accepting a referral, is what we are buying. Arya described it and did not show it.

HIGH

Governing learned rules

Free-text feedback becomes rules. The tool to see and edit “the brain” is still being built. Across 80+ branches, who approves a rule a clinician taught the agent?

MEDIUM

Autonomy boundary

Full-time assignments can write straight to HCHB, and approval is framed as a phase. Our Assist boundary needs approval as a permanent option, per action.

MEDIUM

Proof, not promises

No measured result. Outbound voice and batch outreach were described but not shown live, and the applicant data had low match scores.

MEDIUM

Patient-facing AI

Our clinical operating leaders may limit it. The outbound number, language support and escalation rules all need clinical sign-off.

MEDIUM

Demo data hygiene

During the demo, the presenter's inbox showed what looked like a live escalation from another customer, with a caregiver's name and a patient's initial (0:33:39). Ask how demo and customer data are kept apart. That screen is not included here.

IN THEIR FAVOUR

The whole loop, live

The only vendor to show referral → patient → clinician → HCHB write-back working together, with a suggested time as well as a person.

IN THEIR FAVOUR

Built around the clinician

Text instead of an app, preferences set at onboarding, the clinician's yes before anything lands, and a change-management playbook that matches ours.

What happens now

Agreed on the call

1. **BOTH**
Another virtual session before on-site selection; on-sites the week of 19 October. Colin and Paige to coordinate.
2. **ARYA**
Configure a voice agent for the team to try, and send Paige a Spanish-language text and voice demo.
3. **ARYA**
Next time, show outbound calls and texts to clinicians and the batch-outreach setup.
4. **ARYA**
Show the rules (“brain”) visualization, and come back to the capacity UI with better data.
5. **ARYA**
Check with Anand whether patient messages can use Compassus’s own telephony number.

Recommended for the follow-up

- HCHB: customer count, bi-directional in production since when, and sandbox load-test results for reads and writes at our volume.
- Screen-change detection and a repair SLA. Has the integration broken in production?
- The capacity view on real data: branch supply against expected SOC’s and recurring demand, including held auths.
- Configure approval permanently per action (assign, reschedule, patient message) and show it.
- Rules governance: who approves a learned rule, an audit trail, and a branch-versus-enterprise view.
- Run our shared scenario through a 60-day episode: frequency, LUPA, recert, and two call-outs on one Tuesday.
- A measured result from a named home health customer (Aveanna: home health or private duty?), plus company size and funding.
- How demo and customer data are separated.

Who was there

Compassus

Evan Kramer

Leads Innovation & Operational Excellence

Michael Thompson

VP, Operational Excellence · one year at Compassus, 11 in hospitals

Colin Highland

Initiative lead · PT, ~25 years in home health

Paige Huffman

Senior Project Manager, Operational Excellence

Arya

Kunal Sarda

Co-founder and CEO · led the demo

Anand Chandrasekaran

Principal AI Architect · their HCHB expert

Named, not present

Arun Kalaiselvan (co-founder, CTO); Larissa Licha (VP Product); Courtney Cone (enterprise deployments); Josue Barnes, PhD (enterprise success and strategy)

Built from the Teams recording (2:03:50; the call ends at 1:56:27) and its speaker-labelled Teams transcript, with screens taken from the recording. Where the call skipped a topic (mainly capacity), Arya's questionnaire return (4 Sep 2026) is used and marked "RFP return". Timestamps are into the recording. One screen showing another customer's data was left out.

Prepared for Compassus Operational Excellence · Internal · September 2026